Understanding the National Implementation of a Legalized Abortion Policy: Evidence from Decriminalization in Chile

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Abstract

Abortion laws are subject to frequent change with wide-ranging impacts on fertility and health. Understanding how such legal changes translate into practice is crucial in determining how *de jure* abortion laws map to *de facto* access and use. This study examines the rollout of Chile's 2017 decriminalization of abortion under three specific grounds –life-threatening risk to the woman, fetal unviability, and rape – and its effects on legal access and key usage patterns. We analyze data from administrative records on legal abortion occurring in Chile from January 2018 (shortly following decriminalization) until December 2024, to conduct descriptive analyses examining correlates of usage at the individual and territorial (regional) levels. Our findings show strong correlations between age and the grounds for abortion: the average age is highest for life-threatening risk to the woman, and lowest for rape–with a considerable proportion of adolescents. Geographical disparities are evident: regions with higher rates of sexual crimes also show high requests for abortion due to rape. However, access to abortion is uneven, partly due to geographic variation in conscientious objectors among medical professionals. Despite the increasing access to abortion on the decriminalized grounds over time, various challenges persist that could potentially limit the impact of abortion decriminalization, such as a significant proportion of conscientious objectors, both at the individual and institutional level, and binding gestational age limits in the case of rape.

Résumé

Les lois encadrant l'avortement font l'objet de modifications fréquentes, ce qui entraîne des répercussions considérables sur la fécondité et la santé. Comprendre la façon dont ces modifications législatives se traduisent dans la pratique est essentiel pour déterminer la façon dont les lois sur l'avortement « de plein droit » permettent son accès et son recours « de fait ». Cet article analyse la mise en œuvre de la loi de décriminalisation de l'avortement au Chili en 2017 pour trois motifs spécifiques, c'est-à-dire en cas de viol, de risque vital pour la mère ou de non-viabilité du fœtus, ainsi que les conséquences du recours légal à l'avortement et les principales tendances en la matière. Afin d'effectuer des analyses descriptives des corrélations du recours à l'avortement à l'échelle individuelle et régionale, nous avons analysé les données issues des registres administratifs ayant trait à l'avortement légal au Chili depuis janvier 2018 (soit peu après la décriminalisation) jusqu'à décembre 2024. Nos résultats montrent de fortes corrélations entre l'âge et les motifs d'avortement : les femmes avortant pour des raisons liées aux risques de mortalité ont une moyenne d'âge plus élevée, tandis que les femmes avortant à la suite d'agressions sexuelles ont un âge moyen plus faible, dont une proportion importante d'adolescentes. Certaines différences géographiques sont également marquées : les régions affichant les taux d'agressions sexuelles les plus élevés sont également celles présentant le plus grand nombre de demandes d'avortements consécutifs à des viols. Néanmoins, l'accès à l'avortement varie considérablement, principalement en raison de la présence d'objecteurs de conscience parmi les professionnels de la santé, plus ou moins nombreux dans certaines régions. Bien que l'accès à l'avortement ait augmenté au fil du temps dans le cadre des motifs permettant sa décriminalisation, plusieurs défis pouvant restreindre l'impact de cette même décriminalisation demeurent, notamment le nombre élevé d'objecteurs de conscience, tant sur le plan individuel qu'institutionnel, ainsi que les restrictions strictes en matière d'âge gestationnel en cas de viol.

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Introduction

Criminalization of abortion is detrimental for women's mortality and morbidity, including mental health (Biggs et al., 2017; Boland & Katzive, 2008; Clarke & Mühlrad, 2021; Clarke & Salinas, 2021; Londoño-Veléz & Saravia, 2025). Lack of access to legal abortion is also related to greater socioeconomic hardship and higher probability of intimate partner violence, among other factors (Foster et al., 2022; Roberts et al., 2014). Abortion laws are subject to fierce debate, in particular during the 2020s, with a global conservative backlash against gender also present in Latin America (Biroli & Caminotti, 2020). Nevertheless, recent movements in abortion policies have been observed to both liberalize access and restrict access. Since 2021, abortion has been decriminalized in Argentina, South Korea, and Thailand, while grounds for accessing abortion have been sharply restricted in Poland and the US, among other settings. Understanding how such legal changes are implemented is crucial for determining how *de jure* abortion laws map to *de facto* access and use.

Chile has a history of abortion criminalization. Between 1931 and 1989 abortion was only permitted to save a woman's life. In 1989, the last year of Pinochet's dictatorship, a complete ban on abortion was imposed, promoted by religious and conservative groups from Chile's elite (Pieper Mooney, 2017). Thereafter, women's and feminist organizations sought to decriminalize abortion in some circumstances. More than 11 bills were presented in Parliament, yet each of these was rejected (Maira et al, 2019). During this period, pregnant women could only access illegal and unregulated procedures, risking being reported by health professionals, incarceration, and mistreatment. Criminal prosecution disproportionately affected low-income women, who attended public health facilities (Casas & Vivaldi, 2013). Administrative records of rates of induced abortion did not exist given its clandestine nature, but estimates of the number of abortions carried out in 2014 varied between 60,000 to 100,000 a year (Casas & Vivaldi, 2014). Unplanned pregnancy was estimated at around 50% (JUNJI et al., 2010). The abortion fatality rate did not decrease in this period (Molina-Cartes et al., 2013).

Although the decriminalization of abortion was fiercely resisted by conservative groups and by the Catholic and Evangelical churches, women's and feminist movements actively campaigned for decriminalization. At the turn of the millennium, Chile became a more liberal country, as two socialist presidents were elected. These progressive governments made significant inroads in relation to gender issues, achieving the enactment of a series of laws, such as the passage of divorce (2005), access to the emergency contraceptive pill (2008), the creation of the Ministry of Women and Gender Equality (2015), and the passage of same-sex marriage (2022), among others. Parallel to these changes, the views on abortion in Chilean society evolved. For example, in 1999, 55% of the population believed that 'Abortion should always be banned', yet in 2024 agreement with this statement had dropped to 15% (CEP, 2024).

In 2014 the Chilean President Michelle Bachelet started her second term in office and the decriminalization of abortion was included in her government program (Maira et al., 2019). Abortion became legal in 2017, under three circumstances: when pregnancy threatens a woman's life; when there is a lethal fetal anomaly; or when pregnancy is due to rape. Apart from these three circumstances, abortion remains criminalized. In practice, despite this law, access to abortion remains burdensome. For example, the grounds of risk to life must be confirmed by a physician, while the grounds of lethal fetal anomalies must be confirmed by two physicians. In the case of rape, a social worker and a psychiatrist or clinical psychologist must confirm that the grounds for access are met. Abortion can only be requested up to the 12th week of gestation for women over 14 years old, and the 14th week for those younger than 14 (Maira et al., 2019).

Another obstacle for gaining access to abortion is that the law allows for healthcare professionals directly involved in the procedure to declare conscientious objection. Private health care centers –but not public centers– are also allowed to claim conscientious objection status and therefore refuse to perform the procedure (Baba et al., 2020). Recent estimates indicate that 16% of obstetrician gynecologists (OB/GYNs) in public hospitals declare themselves as objectors on grounds where a woman's life is at risk, 23% on grounds of lethal fetal anomaly, and 46% on grounds of rape (Corporación Humanas, 2023).

In this research note we study the partial decriminalization of abortion in Chile after 2017. Our descriptive results document the reform's implementation, providing insight into a recent national-level adoption of a (restricted) decriminalization of abortion and its population-wide take-up, as well as key challenges limiting access.

Materials, Data & Methods

We accessed administrative records of abortion requests from January 2018 to December 2024. Data were collated from administrative sources which exist from the point of entry of a patient into the healthcare system. Records are kept for all requests, including those not ending in abortion. These data record the exact date of first contact with the health system, date of the procedure, characteristics of the patient including age, type of health insurance, region and municipality of residence, details of the procedure including grounds for which abortion was sought, diagnostic code based on ICD10 codes, type of abortion procedure, gestational period at which abortion occurred, and the hospital or clinic where the procedure was performed.

We added two relevant indicators at the regional level to the dataset we assembled. First, we included data on formal criminal complaints related to sexual violence. Administrative records of criminal reports for sexual assault and other sexual crimes are gathered from public records and are reported by Chile's Ministry of the Interior and Public Security. They are aggregated to a regional-level (Centro de Estudios y Análisis del Delito, n.d.), and we measure them as rates of reporting per 1,000 women aged 15-49[1], for the years 2018-2023. These are matched with rates of abortion, aggregated to the regional-level from our microdata, measured as rates per 100,000 fertile-aged women, during the same period. Secondly, we added information on OB/GYNs who are registered as conscientious objectors, as a proportion of all their peers working in the public health system, at the regional level and by each of the three abortion grounds. This figure was published by Corporación Humanas (2023), based on Freedom of Information requests made to the Ministry of Health.

We first provide a basic breakdown of the law's impact on abortion, focusing on the total number of abortions requested and rate of pregnancy terminations, under the three permitted grounds. We then plot the distribution of abortions that were completed, according to women's ages and gestational stages. We next describe differences in the health insurance of women who request or undergo an abortion. Finally, we delve into abortions requested on grounds of rape, relating it to rates of sexual assaults and crimes, and to the proportion of OB/GYNs who are conscientious objectors.

Results

In Chile, between 2018 and 2024, a total of 5,991 legal abortions were requested. We observe an increase of from 668 abortions (in 2020) to 1,059 abortion requests (in 2024) per year. The grounds with most requests is lethal fetal anomalies, accounting for 51% of requests, followed by risk to women's lives (34%). Requests on the grounds of rape represent 23% of all requests. Notably, as Panel A in Figure 1 shows, requests for abortion due to rape have increased significantly, rising from 17% in 2018 to 36% in 2024. Panel B in Figure 1 shows that requests have increased since 2018, except during 2020, in line with reduced capacity of the health-care system during the first year of the COVID-19 pandemic.

Panel A in Figure 2 depicts the age of women who requested abortions, according to the grounds of the request. For the request on grounds of risk to the woman's life, the age distribution of women is relatively normal, with a mean of around 31 years. The requests for abortion on the grounds of lethal fetal anomalies increase for women in their twenties and resemble a plateau-shaped distribution during their thirties, decreasing afterwards. This is consistent with the fact that rates of fetal unviability increase with maternal

1 These rates must be interpreted with caution, given that underreporting of sexual crimes is common. In the US, it has been estimated that one of three sexual assaults is reported (Passmore, 2023). In Chile, survey data indicates about two thirds of all crimes (not only sexual crimes) are not reported (Gamarra et al., 2024). The rates we use are the number of complaints made to the police regarding sexual crimes (different from trials or arrests related to sexual crimes).

age (Fretts et. al., 1995), explaining a rightward skew in the age profile of abortion requests under this grounds (observed in Figure 2, Panel B). By contrast, women who request an abortion because of rape are much younger. Their distribution is skewed; the modal age was 13 years, minimum age 11 and maximum age 46. Regarding gestational stage, shown in Panel B, requests tend to be made throughout the pregnancy in the case of risk to the woman's life, though 50% of requests are concentrated between weeks 15 and 22. Abortion procedures for cases of lethal fetal anomaly tend to take place in the second and third trimesters. Requests on grounds of rape are concentrated in the first trimester because the legal framework states that they cannot take place after 12 weeks of gestation, except for girls under age 14, who can make these requests up to week 14.

Another relevant topic in which there are differences among women requesting an abortion is their health insurance, as shown by Figure 2, Panel C. Chile has a mixed healthcare system, with individuals electing for coverage from the public insurance provider (FONASA) or from private insurance companies (ISAPREs), which is generally more expensive. A small percentage of the population is insured through a system reserved for the Armed Forces or those who have no insurance. As a benchmark, the last column in this panel shows the national distribution of health insurance for women. Panel C in Figure 2 suggests a negative income gradient in requests where public insurance is viewed as a proxy of income. The largest proportion of requesters (86%) are FONASA users, and only 11% of women requesting an abortion are covered by ISAPREs. Even though these figures follow the same direction as the national distribution of health insurance for women in fertile ages --79% FONASA, 14% ISAPREs (Ministerio de Desarrollo Social, 2020)-- there is a slight overrepresentation of FONASA users and a slight underrepresentation of ISAPRE patients among women who request an abortion. The overrepresentation of FONASA patients is more marked when considering requests based on risks to the women's lives and rape (90% of these requests are to FONASA users). Private insurance companies are observed to cover more abortions requested on the grounds of fetal unviability (17% of those requests are from ISAPRE users). By contrast, only 5% of abortion requests on the grounds of rape are to ISAPRE users.

Beyond the relationships between abortion requests, usage, and individual characteristics, we observe regional correlations that suggest broader social determinants affecting access to abortion services, though these also point to substantial unobserved heterogeneity in access by area. These spatial correlations align with the country's considerable regional, economic, and cultural diversity. Figure 3 illustrates regional rates of abortion due to rape (Panel A), reported sexual crimes (Panel B), and the proportion of OB/GYNs working in public hospitals registered as conscientious objectors (Panel C). Positive correlations exist between rates of complaints for sexual crimes and abortion requests. We observe substantial variation in the proportion of conscientious objection. Importantly, some regions of Central Chile, such as O'Higgins (VI), Maule (VII) and Ñuble (XVI), have high rates of complaints for sexual crimes and abortion requests, but less than half of OB/GYNs working in public hospitals are willing to perform an abortion based on these grounds. This figure underscores how problematic restricted access to abortion continues to be for women in several regions of Chile, even several years after partial decriminalization.

Across the country, there is a considerable proportion of medical professionals who register conscientious objector status, particularly among OB/GYNs. Most of these physicians object to performing abortions requested on the grounds of rape. At the national level, 46% of OB/GYNs working in public health care facilities are conscientious objectors, which is approximately two times higher than the proportion of conscientious objectors for other grounds (Corporación Humanas, 2023). Panel C in Figure 3 shows that in nine of the 16 regions of Chile more than half of the doctors working in public health care facilities are conscientious objectors, reaching a maximum of 74% in the region of O'Higgins (VI). This figure is vastly different from that in the capital (Metropolitan) region or the region of Coquimbo (IV), where only 36% and 29% of OB/GYNs are registered as conscientious objectors, respectively. Note that in the case of rape, access time for the procedure is particularly acute given that the procedure can only be requested until 12 weeks of gestation (14 weeks for girls aged 14 or younger), suggesting that a lack of access to medical procedures in a timely matter may result in an inability to complete the procedure.

Discussion

The landscape of access to abortion in Latin America is diverse, ranging from severe restriction in Haiti, Honduras, El Salvador, and Nicaragua, to access on demand in Colombia, Argentina, Uruguay, and Mexico (Roth, 2023). Following the decriminalization of abortion under three grounds, Chile is in between these poles. In this research note, we use administrative records to describe key trends in abortion access in Chile after that reform. We show that the requests have increased over time, even though the number of requests per year is still relatively low, reaching only 1,059 abortions in 2024. This figure is far below the estimated quantity of induced abortion in Chile (around 60,000 per year according to Casas and Vivaldi, 2014), which is understandable, given that the reform only decriminalized abortion under three very specific grounds.

Demand for abortion was highest in cases related to fetal unviability, followed by risk to the mother's life, with the lowest demand for rape cases. However, requests due to rape have been increasing in recent years, both in absolute and relative terms, evidencing that to date provision of legal abortion due to rape likely does not account for the real demand for it. This request was the most controversial when the law was discussed. It was expected that abortion on the grounds of rape would be the main cause (Cardenas, 2025), however our results show the opposite.

We found differences in the age distribution of women requesting an abortion, depending on the grounds for which the procedure was requested. Most strikingly, rape-related abortion is concentrated among very young women, with a mode of 13 years-old, whereas for abortions requested on the other grounds, the age distribution follows patterns that are more in line with prior medical literature (Fretts et al, 1995). The distribution of requests according to gestational stage also follows what could be expected, either as discussed in previous literature (Creanga et al, 2015) or due to the limitations imposed by the law itself. In addition, we found evidence that suggests an overrepresentation of users among women with public health insurance, who on average have lower incomes.

Our results also speak to important differences in access at the regional level. Despite Chile's highly centralized nature and the reform that decriminalized abortion based on three requests being a national policy, the access to and implementation of the law vary substantially across regions. Regions with higher rates of sexual crimes tend to have high rates of abortion requested on the grounds of rape. Note that even though the data we use for measuring abortions covers all requests, our data is likely limited with regard to sexual crimes, which tend to be underreported. Many of these same regions also have limited access to providers, as implied by their high proportions of conscientious objectors. Sub-national studies are essential for identifying local factors, such as the unequal distribution of medical resources, the availability of non-objecting professionals, and the social and cultural dynamics that influence service provision.

Our results imply there is a large, unsatisfied demand for abortion on other grounds, when comparing the total of legal abortions requested per year with the estimated figure of induced abortions (Casas and Vivaldi, 2014). But these results also suggest obstacles for accessing abortion on the decriminalized grounds -- particularly notorious on the grounds of rape. Recent research suggests that conscientious objection is one of the main barriers to explaining why actual abortion numbers have been much lower than projected (Montero et al., 2023). Chile is among the few countries allowing institutional objections, in addition to individual objections (Michel and Repka, 2024). Chile is similar to Argentina in that conscientious objection is limited to health professionals directly participating in the procedure (mostly physicians). However in Argentina it seems that there are clearer protocols to follow in cases where conscientious objection is invoked, such as arranging transportation to a different center within a given number of days (Binstock et al, 2024). Furthermore, Chile also allows institutional conscientious objection. In Mexico, on the other hand, legislation regarding conscientious objection has been before the Senate since 2023, but no advances have been made (Barrientos Nieto, 2023), which is similar to the situation in Colombia, where, even though abortion on demand is allowed until week 24, in practice there are no protocols to follow when someone

claims conscientious objection (Quiroga, 2024). Chile has very clear regulations on conscientious objection, but these regulations comparatively facilitate the providers -individuals and institutions- in claiming objection, hindering women's choices.

Other obstacles are healthcare professionals' lack of training, distance and connectivity issues, and lack of public campaigns (Casas et al., 2023). Potential policies in the future can address these issues, through increasing the training of medical staff on the various procedures to perform an abortion and media campaigns that increase knowledge about the rights women have under the reform. Continuing the debate about abortion is necessary to increase access, if not allowing for abortion on demand, at least restricting conscientious objection on the permitted grounds and creating protocols that include transportation and lodging for those in need when a referral to a different health center is made.

Additionally, there are restrictions imposed by the legal framework, such as limitation of gestational stage for the grounds of rape, the fact that abortion should by performed at secondary or tertiary (not primary) care centers, and the need for professional confirmation for the grounds of lethal fetal anomaly and rape (Muñoz et al, 2021). The requirement that rape be reported to the public prosecutor's office has also been reported as a perceived barrier to abortion (Ramm et al., 2020). Addressing limits to the law and strengthening regional access would likely ease the burden on women seeking abortion through official channels, which may have strong positive impacts on women's health and agency, as documented in the case of other contraceptive reforms in the country (Clarke and Salinas, 2021). In Latin America, there is great variability in access to legal abortion, but the concern about the implications of restricted access to abortion for women's health and wellbeing are abundant across the region in academic circles. Recent policy movements in several countries in the Americas suggests that access to abortion is still far from universal in the region. In this context, sustaining and improving access to reproductive rights, including abortion, is a key demographic challenge faced in the Americas.

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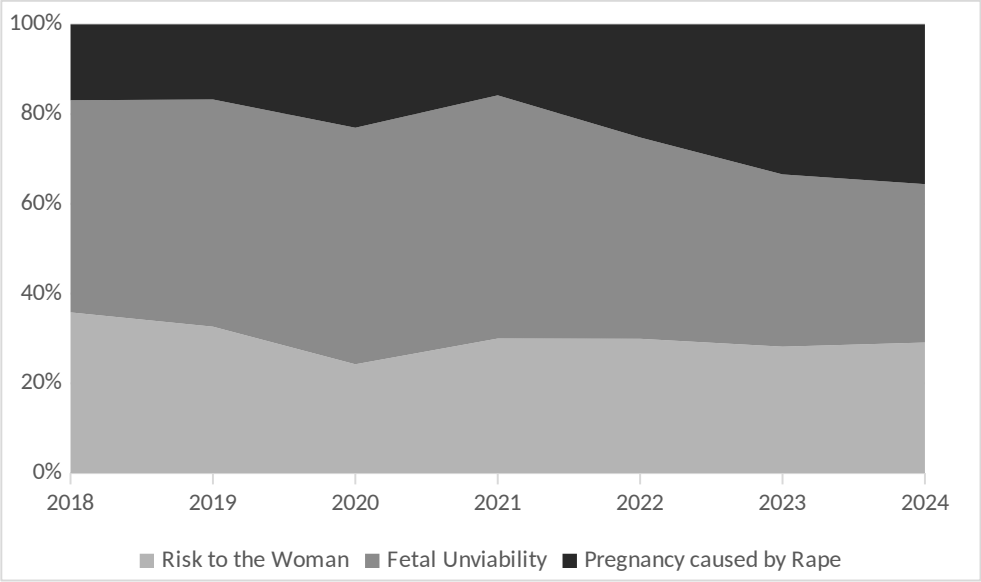
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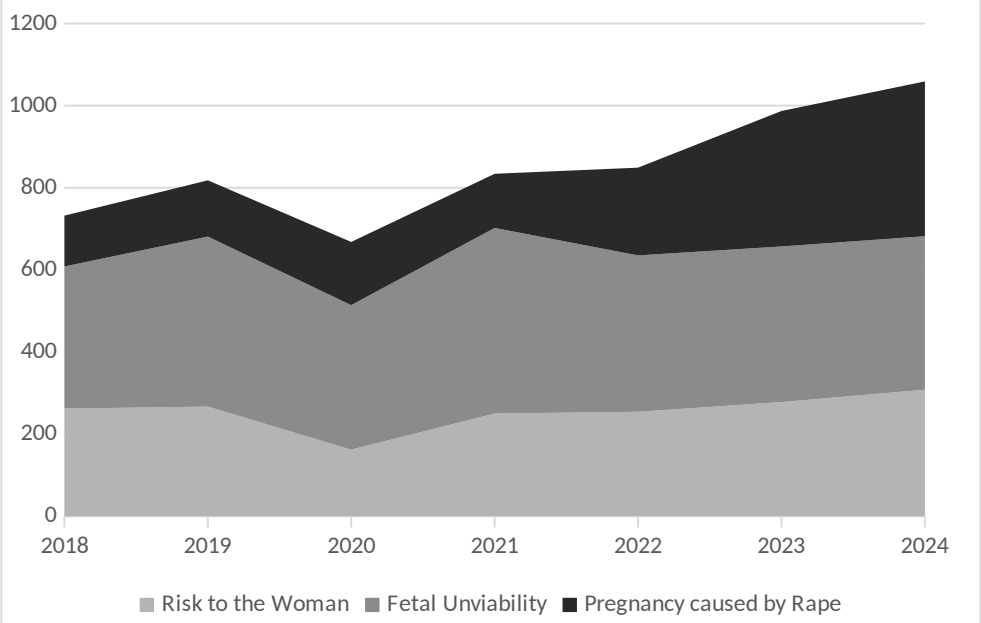
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Figure 1: Abortion Requests by Cause and Time

Panel A: Percentage of requests classified by grounds each year



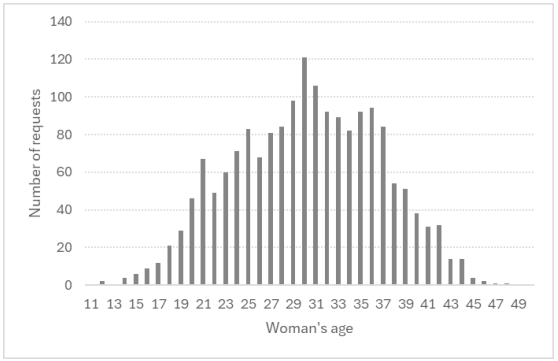
Panel B: Number of requests classified by grounds each year



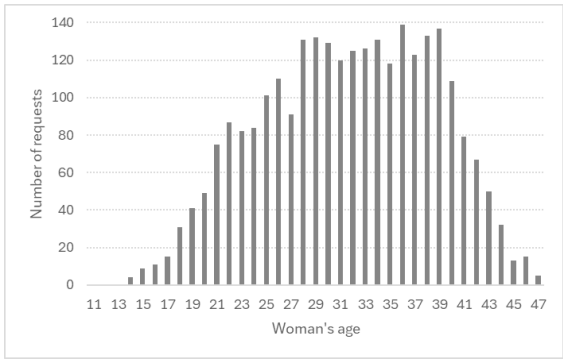
Notes: Most of the requests end in abortion, approximately 83% of requests on the grounds of women at risk, 82% if there is fetal unviability and 92% if the request is on the grounds of rape.
Figures are calculated from anonymized individual level administrative records made available from the Department of Health Statistics and Information (DEIS) of Chile's Ministry of Health.

Figure 2: Characteristics of Requesting Women – Age, Gestational Stage and Health Insurance Coverage

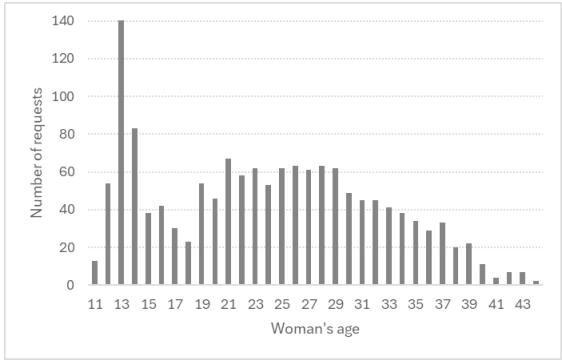
Panel A: Women's Age (in years)



(a) Risk to the Woman

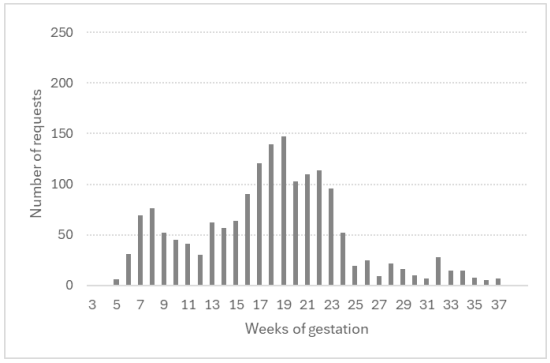


(b) Fetal Unviability

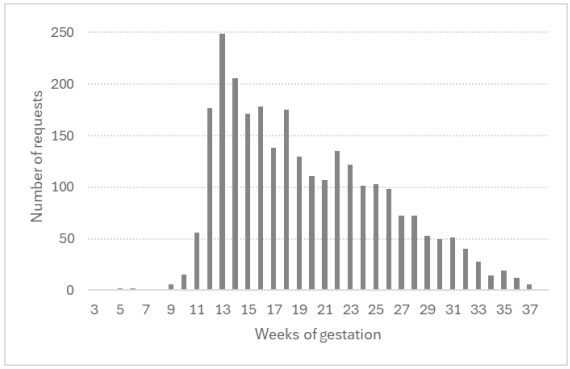


(c) Pregnancy Caused by Rape

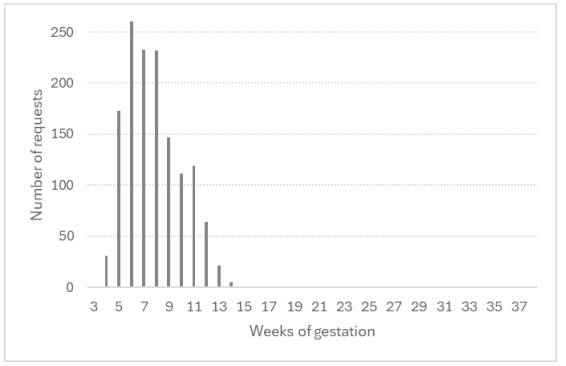
Panel B: Gestational Length (in weeks)



(d) Risk to the Woman

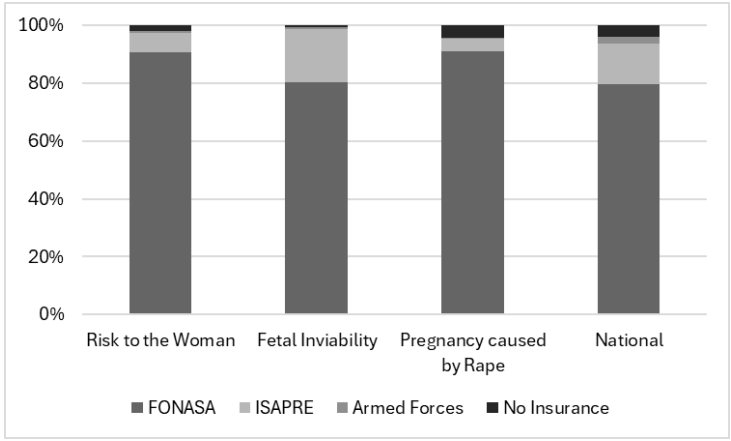


(e) Fetal Unviability



(f) Pregnancy Caused by Rape

Panel C: Health Insurance (percentage)

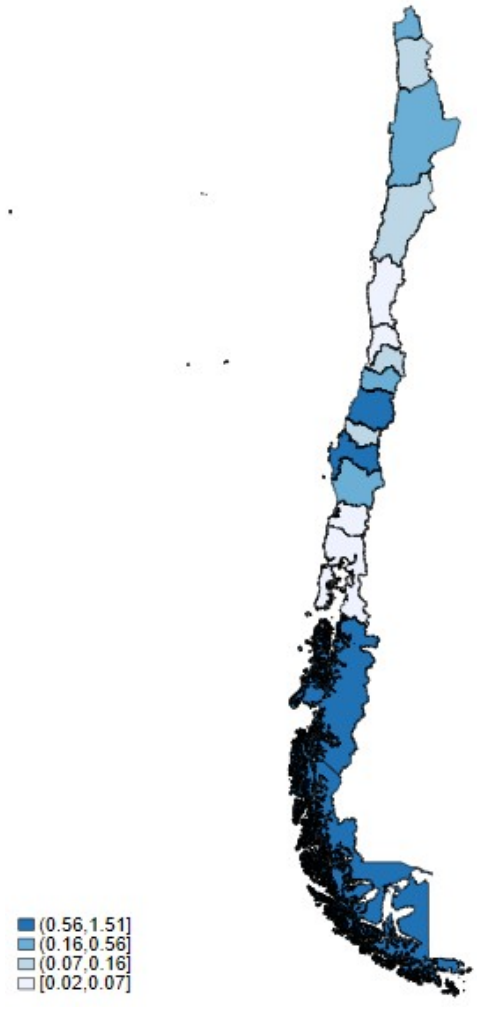


(g) Type of Health Insurance as a % of total requests on that ground, and national-level.

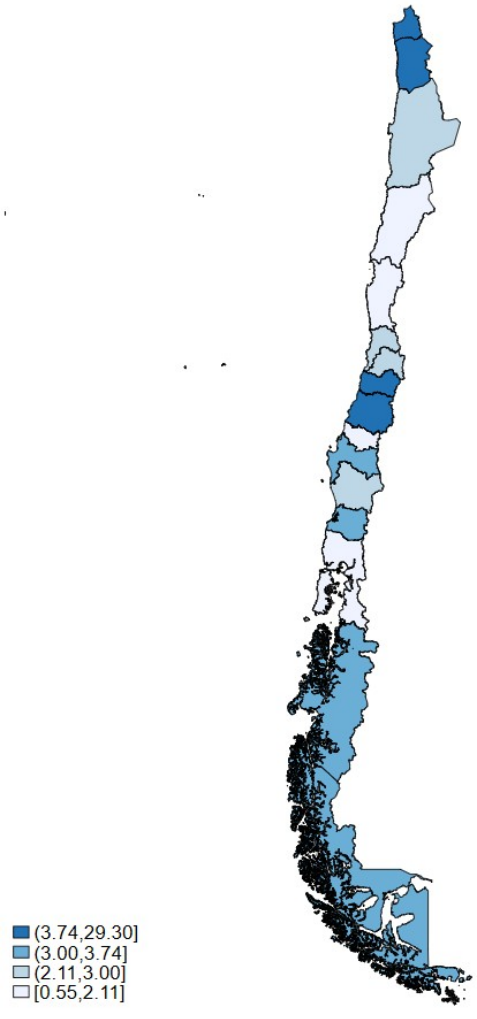
Panel A: Woman's Age (in years)

Notes: All histograms present total frequencies of the number of abortions carried out from January 2018 to September 2024, based on anonymized individual level administrative records made available from the Department of Health Statistics and Information (DEIS) of Chile's Ministry of Health. Mother's age (panel A) refers to her age in years as recorded at the time of first contact. Gestational length (panel B) refers to the pregnancy's estimated gestational length at the time of the procedure. Health insurance (Panel C) provides comparable rates of health insurance coverage among all Chilean women in 2022, compiled from Data Social Ministerio de Desarrollo Social y Familia (2022).

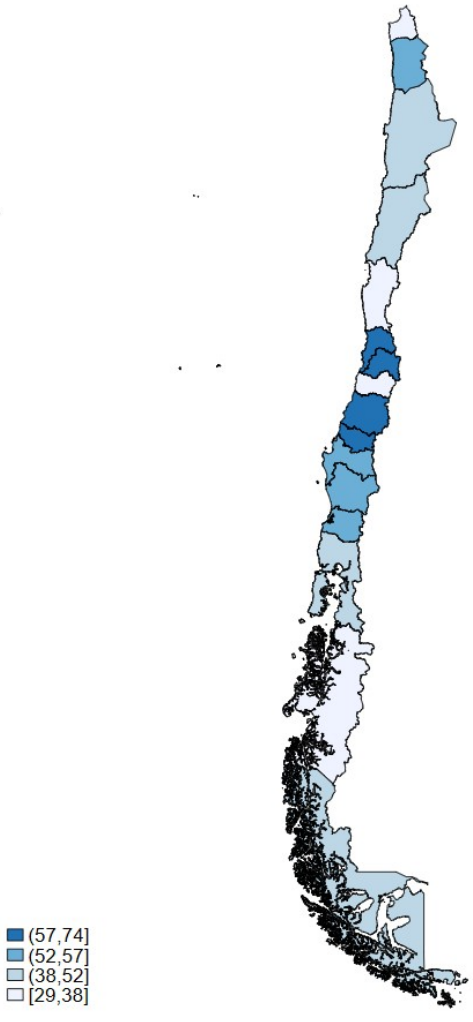
Figure 3: Distribution of Requests Related to Rape and Criminal Complaints of Sexual Crimes



(a) Abortion Requests Owing to Rape



(b) Complaints of Sexual Crimes



(c) Proportion of OB/GYN working in public hospitals who are registered as CO.

Notes: Panel (a) plots the mean rate of abortion requests where the pregnancy was caused by rape per 100,000 fertile aged women (2018 to 2024), panel (b) plots the mean rate of criminal complaints of sexual crimes per 1,000 fertile aged women (2018 to 2023) and Panel (c) plots the rate of OB/GYNs working in public hospitals who in 2023 are registered as conscientious objectors (CO) relative to the total number of OB/GYNs in public hospitals. Panel (a) is plotted based on DEIS's administrative records, Panel (b) is plotted based on administrative records covering all criminal complaints published by Chile's Interior Ministry, and Panel (c) is plotted based on Corporación Humanas (2023).